



OFFICIAL

Outpatient Clerical Resource Guide

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1. Purpose

This document serves to provide a comprehensive overview of the clerical and administrative procedures and booking rules, applicable within the Armadale Kalamunda Group (AKG) Outpatient Clinics.

Upon commencement, all outpatient staff are required to complete relevant systems training and become familiar with webPAS user guides and site-specific policies and procedures. This document is intended to support that onboarding process by outlining key operational practices.

2. Applicability

This Resource Guide is intended for all clerical team members within the Armadale Kalamunda Group Outpatient, Ward, and Waitlist areas who are responsible for capturing and recording outpatient data in webPAS.

3. Clerical Business Rules

3.1 Customer Service

Customer service is a fundamental aspect of the Outpatient Clerical role. As the first point of contact for patients, outpatient clerks play a key role in shaping the patient experience. It is essential that patients are acknowledged promptly upon arrival. If immediate assistance is not possible, staff should courteously inform patients that they will be attended to as soon as possible. The Clerical Team is also responsible for managing various communication channels. Professionalism and consistency in handling all forms of communication are critical to ensuring both efficient service delivery and a positive patient experience.

3.2 Patient Master Index

The Patient Master Index (PMI) is a comprehensive database containing demographic information for individuals—primarily Western Australians—who have attended a public health facility. This data is maintained within webPAS (web-based Patient Administration System), the centralised hospital patient management system used by all Western Australian Public Health Service Providers (HSPs). With over 3 million registrations, staff should initially assume that a patient is already registered in the system, particularly those born after 1980.

Each new registration generates a unique patient identifier known as a U/R number, consisting of seven numeric characters prefixed by a letter (e.g., J1234567). This identifier is used across all public hospitals attended by the patient and, under normal circumstances, should be the only number assigned to that individual.

Maintaining a single Unique Medical Record Number (UMRN) per patient is critical to ensuring continuity of care. All clinical interactions and healthcare data must be recorded under this single identifier to ensure accurate, accessible, and consolidated patient information across all public hospital sites.



The PMI standards are available on the '?' help button in webPAS and via the following link [PMI Standards](#)

3.3 Referrals

Outpatient services are provided only upon receipt and acceptance of a valid referral. Patients must not be seen for treatment unless a referral has been received and formally accepted by a clinician.

All referrals received at Armada Kalamunda Group (AKG) are recorded and electronically uploaded to eReferrals by the Referral Clerk. This centralised system ensures that clinicians can access and action referrals efficiently from a single platform.

Referral Clerks play a key role in the gatekeeping process, ensuring that accepted referrals fall within the AKG postcode catchment area and redirecting to appropriate tertiary facilities if required.

If a referral is received directly in your clinic via mail, fax, or email, please arrange for it to be collected or forward it via email to: AHS.Referrals@health.wa.gov.au



REFERRAL RECEIVED DATE

In accordance with Department of Health (DoH) guidelines, the original referral received date must not be changed under any circumstances, regardless of where the referral is actioned—whether at the initial site or after being transferred to another facility. Preserving the original received date is essential to ensure accurate reporting of outpatient waiting times and compliance with statewide referral management standards.

3.3.1 GP and Specialty Room Referrals (External Referrals)

Referrals from General Practitioners (GPs) and specialist rooms are considered external referrals and are submitted directly to the Central Referral Service (CRS)—an external agency responsible for managing referral distribution.

CRS allocates referrals to hospitals based on the patient's residential postcode catchment and the level of care required. Once allocated to AKG, the site Referral Clerk receives these referrals via SharePoint and uploads them to eReferral for triage and clinician access.

3.3.2 Internal and Intra Hospital Referrals – eReferral sites

Referrals originating from other specialties or departments within AKG, or from other public hospitals, are classified as internal or intra-hospital referrals. These must be created and submitted via eReferral.

As these referrals are internal to the public health system, they are sent directly to the relevant clinicians for triage within eReferral, streamlining the process and ensuring timely access to care. Note: This process does not apply to referrals from non-public hospitals (e.g., Midland SJOG, Joondalup Health Campus, and some WACHS sites). For guidance on these, refer to section 3.3.3.

3.3.3 Intra Hospital Referrals – Non eReferral sites

Referrals received from non-public hospitals—including St John of God Midland, Joondalup Health Campus, and certain WACHS sites—are not processed through eReferral. These referrals must be emailed or faxed directly to: AHS.Referrals@health.wa.gov.au This ensures appropriate processing and routing within the AKG outpatient services.

3.3.4 Triaged Referrals

Referrals will not appear in webPAS until they have been triaged by the relevant Specialty Gatekeeper.

The Gatekeeper (Clinician) reviews each referral to ensure it meets service and catchment criteria, assigns a category code, and provides booking instructions for the clerical team in the 'Clerk Notes' field.

Any information entered in the 'Clerk Notes' field must be copied into the webPAS appointment comments once an appointment has been scheduled.

For urgent referrals requiring an appointment within 7 days, direct communication with the specialty team via telephone is required prior to submission. If accepted, these referrals are sent directly to the site (either the referral office or clinic) and not via CRS.



Referrals received by the hospital are to be triaged by the relevant clinical gatekeeper within the following timeframes:

- Immediate referrals – initiated by referring clinician, telephone discussion
- Urgent referrals < 1 working day
- Semi-urgent/Routine referrals < 5 working days

Referrals are triaged as per priority:

Category 1	0 - 30 days	Appt must be booked on receipt of referral to be seen within 30 days
Category 2	30 - 90 days	Book within 90 days in date order of when referrals are received
Category 3	90 - 365 days	Book within 365 days. Monitor wait times and book appt when available

Category 1 referrals must be booked within the 30 days from the date the referral is received. If unable to book within this period escalation to the Clinician is required. If an ongoing issue, escalation to the OP Clerical Coordinator and OP Manager is to occur.

Category 2 and 3 booking timelines are used as a general guideline for booking appointments. Appointments may be booked earlier if the clinic has capacity.

3.3.5 Referral Management Guidelines

To support timely and complete patient care, it is essential that patients have only one active referral per issue per specialty within the health service. (Note: Obstetrics is an exception to this rule.)

- Cross-Site Checks:
Verify that the same referral does not exist at other health sites before proceeding.
- Wait Times Integrity:
The original referral date and wait time must always be retained, regardless of changes.
- Inappropriate Referrals:
If a referral has been incorrectly accepted by the Gatekeeper and is not suitable for your service, refer it back to the clinician for review and appropriate action.

3.3.6 Duplicate Referrals

If a second referral is received for the same presenting issue:

- The Gatekeeper will triage and determine whether the referral priority should be upgraded.
- If no change is required, the clinician should select “No appointment required”, which prevents the referral from flowing into webPAS.
- If the clinician forgets to do this and a duplicate referral appears in webPAS with no changes, the second referral must be closed.

Multiple Referrals Same Specialty

Patients may have multiple referrals under a specialty only if the referrals are for different clinical issues which require different clinicians to treat them, i.e. Hips vs Shoulder.

If the clinician can treat both issues, please add the 2nd presenting complaint to the first waiting referral and close the duplicate referral. WebPAS referral presenting complaint will then have 2 issues listed.

Clinicians are to review all referrals for a patient in eReferrals when undertaking initial consults (closed or open).

3.3.7 Referral Escalation or Priority Change

Referral Priority Upgrade/escalation:

- If the clinician upgrades the referral priority during triage, the updated referral will flow into webPAS as a second referral with a more urgent category.
- If the patient has not yet been booked, the clinician can update the original referral priority in eReferral, which will then sync with webPAS. The duplicate referral can then be closed.
- If the patient has already been booked, the referral priority in webPAS cannot be changed. In this case:
 - The clerk must document the category change in the NOTES tab of the webPAS referral.
 - Update the preferred appointment date if the appointment cannot be scheduled immediately.
 - If re-triage results in a category change and the patient is now over the boundary, the clerk must book the appointment as soon as possible.



The original referral date must always be retained.

3.3.8 Active Life of Referral

Referrals for outpatient services remain valid for a single opinion or course of treatment for a defined period starting from the date of first attended appointment with the outpatient service.

External Referrals (e.g. GP, Nurse Practitioner)

- Validity: 12 months from the date of first attended appointment
- Exception: Can be indefinite if specified by the referrer

Internal Referrals (Clinician to Clinician within WA Health)

- Validity: 3 months from the date of first attended appointment

Specialist to Specialist Referrals

- Validity: Either:
 - A single course of treatment, or
 - 3 months from initial consultation – whichever is lesser

Indefinite Referrals

- Criteria:
 - For chronic conditions beyond three or twelve months
 - Must detail the clinical condition
 - Must indicate ongoing specialist management is required
- These include:
 - PSY CTT (Psychiatry)
 - RHE (Rheumatology)
 - CONT (Continence)

3.3.9 Updating webPAS Referrals after triage

- Once triaged in eReferral, the referral automatically flows through to webPAS.
- Referral will be visible on the specialty Action List.
- Review daily to identify and update NEW referrals.

Steps to Process a New Referral

1. Open the referral from your Action List in webPAS.
2. Click the link to view in eReferral.
3. Review referral content:
 - Identify presenting complaint
 - Determine claim type
 - Determine if preferred date needs updating based on booking instructions/notes to clerk.
4. In webPAS:
 - Click "Update"
 - Enter presenting complaint and claim type
 - Check Medicare number
 - Update preferred date if patient to be seen sooner than end of category date, i.e. in 2 weeks not 4, etc.
5. Verify and update patient demographics if new or different information is provided.

Financial Eligibility

- Refer to:
 - Financial Election Tip Sheet with Flowchart
- If patient is not eligible for Medicare:
 - Notify AKG Finance Team
 - Finance will advise patient of current fees and charges
 - Refer to OV/OS Flowchart.

3.3.10 Discharging / Closing Referral

When to Close a Referral

- Close the referral when the clinician confirms treatment is complete.
- Most referrals are closed during appointment processing.

Before Closing a Referral

- Cancel any future appointments or appointment requests linked to the referral.

Why Closure Is Important

- Ensures accurate reporting of patients awaiting care.
- Prevents inactive referrals from remaining on Action Lists.

Referrals without future booked appointments or appointment requests must be closed.

3.3.11 Reactivating referrals

Referrals should only be reactivated under the following circumstances:

Valid Reasons to Reactivate

1. Referral closed in error:
 - Patient's treatment is not complete or has not occurred.
2. Ongoing care required (SOS discharge) – Refer section 13.1
 - Within 3 months of discharge from clinic.
3. Patient contact received:
 - After a non-contactable letter was sent or a waitlist audit was completed.

Next Steps After Reactivation

- Book an appointment immediately, or
- Create an appointment request.

If Unsure

- Seek advice from clinical and supervisory staff before reactivating.



Once reactivated the referral will display a status of 'Active', not 'Waiting' and **will not be visible** on the 'Action List'.

4. Booking Process

All outpatient appointments **must**:

- Be linked to a valid referral
- Be scheduled based on:
 - Priority category assigned
 - Referral receipt date and preferred date
 - Triage category timeframe
- New appointments: allocated by referral preferred date within each triage category
- Follow-up appointments: allocated by preferred date

Patient Notification

- Appointment notifications must be sent via:
 - Letter
 - Phone or SMS (if appointment within 2 weeks)

Interpreter Services

- If required:
 - Interpreter must be booked at time of appointment creation
 - webPAS appointment comments must include interpreter booking details
 - Refer to: W:\HIMS\AKH\General\Interpreters

4.1 New Appointments

New visit type is to be utilised for the initial visit booked against a referral.

When booking the initial appointment clerical staff should:

- Add relevant approved codes from the list below or comments to highlight important information.
- This information is to be kept to a minimum and must not include clinical information.
- These comments will be visible on clinic lists, aiding communication and preparation.

Approved Clinic Communication & Reminder Codes			
XR	X-ray	PW	Pathology
US	Ultrasound	TC	Time Critical
CH	Chaperone	EQ	Equipment (e.g., Moon boot)
HR	High Risk	TPC	Telephone Call
TH	Telehealth	F2F	Face to Face or Client Present

4.2 Follow Up Appointment

Follow up visit type is utilised for subsequent outpatient appointments, including the first outpatient appointment for patients referred from same specialty inpatient ward (same hospital).

Booking Instructions

- Provided by the clinician after the first appointment via:
 - Clinic list
 - Completed outcome slips
 - Refer to Appendix 1 for details.

Clinician Input

- Clinicians will specify:
 - Timeframe for the next appointment (this becomes the preferred date).
 - Whether the appointment is time critical, or the patient is high risk.

Appointments Beyond 3-Month Horizon

- Must be added to the Action List by creating an Appointment Request.
 - Example: 3 months from 3rd June = preferred date of 3rd September.

Documentation in webPAS appointment comments

- Transcribe follow-up booking instructions from the outcome slip into webPAS appointment comments.
- Include any specific instruction codes relevant to communication and action.
- Clinical information must not be recorded in appointment comments.
 - Refer to Section 4.1 for guidance.

4.3 Overbook Appointments - New and Follow Up

When to Use Overbook

- Only when all appointment slots for a clinic session are fully booked and no further slots are available.

Clinician Approval

- Mandatory before adding any overbooked appointment.
- Document approval in appointment comments (e.g., "Overbook approved by Dr. Smith").

4.4 Slot Conversion Rules

- Converting slots from New to Follow-Up (FOL):
 - Must not result in Overbook appointment types as is within the normal clinic session
 - Must not cause waiting referrals to be going over boundary.

4.5 Reserving Slots

- Reserving slots should not lead to overbooked appointments if added at a different time.
- Use standard New or FOL appointment types instead.
- If clinician requests standard reservations, i.e. last appointment every clinic, please liaise with Outpatient Manager to follow up.

4.6 Time Critical Appointments

Definition

- Indicates the patient's care is time sensitive or the patient is high risk.
- Appointments must not be rescheduled without prior consultation with clinician or postpones to a distant date (e.g., not 3 months later).

Clinician Instructions

- If deemed Time Critical, this will be clearly indicated on the outcome slip.
- The patient must be seen within the specified timeframe.

Booking Requirements

- Include 'TC' in:
 - The Presenting Complaint field.
 - The webPAS appointment comments.

Clinician must be notified if

- No close appointment date is available
- Patient requests a long-term reschedule or cancellation of care.

4.7 High Risk Patients

Definition

- High Risk patients are those with urgent medical history or risk of poor outcomes/abuse.
- Their care requires timely follow-up and action.

Appointment Changes

- Any changes to HR patient appointments must be within 2–3 weeks.
- All changes (rescheduling, discharge requests, or patient declining further care) must be escalated to the clinician for review prior to amending.

Documentation Requirements

- **Outcome Slip:** HR status is indicated.
- **webPAS:**
 - Include “HR” in the **Presenting Complaint** field.
 - Transcribe “HR” into the **webPAS appointment comments**.



Risk Alerts may be used to identify patients who are vulnerable in their home environment, present significant medical risks, or pose potential safety concerns for staff.

[Risk Alert Notification and Management Plan AKMR ALERT4](#)

4.8 Outpatient Follow-Up Post Ward Discharge

- If an inpatient requires outpatient follow-up post discharge, clinical ward staff must complete an eReferral to the relevant clinic for triage and processing.

- Patients must meet the clinic's service criteria to be accepted

For patients seen in clinic prior to admission

- For patients who were seen in the clinic prior to admission (e.g. booked surgery), follow-up appointments post discharge should be arranged based on one or more of the following:
 1. Receipt of a ward eReferral.
 2. Review of theatre lists and/or discharge lists with instructions from the ward.
 3. Direct contact from the patient requesting follow-up.

Important Notes

- Do not refer patients to the clinic solely for results follow-up related to inpatient care. These results should be managed on the ward.

4.9 Telehealth Appointments

Telehealth Appointment Types

- Telephone – standard phone consultation.
- Video Call (single appointments) – conducted via Healthdirect Video Call.
- Avaya (group appointments) – used for group telehealth sessions.

Booking Locations

- Telehealth at Non WA Health Site (TH at Non WA)
Use this when the patient is at home during a video consultation.
- Telehealth at WA Health Site. Use this when the patient is at another hospital or health centre (e.g. Boddington Health Centre) and is being assisted by a clinician or nurse.

Patient Contact Details

- Always request the patient's email address and enter it into webPAS to enable sending of video call instructions.

Reference Resources

- [Telehealth Process Guides](#)
- [AKG Telehealth intranet hub page](#)
- [Virtual Care Toolkit \(health.wa.gov.au\)](http://health.wa.gov.au)
- [Avaya Workplace Training Resources \(health.wa.gov.au\)](http://health.wa.gov.au)
- [Video Call tools - including Share Screen - Healthdirect Australia](#)

4.10 Chart Only Appointment

Purpose

- Chart Only appointments are classified as non-client events indicating the patient will not be in attendance.
- They are used for clinicians to review patient records, results, or monitor progress without the patient being present.

Booking Requirements

- Booking a Chart Only appointment ensures the patient's health record is available in the clinic at the time of review if applicable.
- The Service Delivery Mode must always be set to **'Other'**.

Restrictions

- Chart Only appointments must not be used with service delivery modes such as:
 - Phone Call
 - Video Call
 - Client Present
- These modes indicate patient attendance, which is not applicable for Chart Only events.

Appointment Outcomes

- The outcome must reflect the actual patient journey, such as:

- Reappoint
- Discharge from clinic
- “Chart Only” is not to be utilised as the outcome.

Funding and Activity

- Chart Only appointments are not counted in clinic activity and do not attract funding.
- Therefore, it is essential that Chart Only is not used for any appointment where the patient is present.



If the Service Delivery mode is incorrect for a Chart Only appointment patients will receive an SMS appointment notification advising to attend the appointment which is incorrect.

4.11 Multidisciplinary Case Conference – approved MCC clinics only

Definition

- MCC appointments involve the clinical team (minimum of three clinicians from different professional groups) discussing care plans for multiple patients.
- Patients are not present during MCC appointments.

Booking Requirements

- MCC appointments must be booked using Chart Only (C/O) slots in an approved MCC clinic only.
- The Service Delivery Mode must be set to ‘Multidisciplinary Case Conference’.
- Not to be used ad hoc in other clinics / areas.

Appointment Outcomes

- The outcome must reflect the actual patient journey, such as:
 - Reappoint
 - Discharge from clinic
- “Chart Only” is not to be utilised as the outcome.

4.12 Converting Chart Only Appointments to a Funded Service Event

When to Convert

- If a clinician contacts a patient as a result of a Chart Only (C/O) appointment and the discussion meets the service event criteria the appointment can be converted to a valid patient event.

Process

- The clinician must notify the clerk to initiate the conversion.
- The clerk is responsible for updating the appointment in webPAS.

Conversion Requirements

- Update the Visit Type to reflect the correct classification:
 - New or Follow-Up
- Update the Service Delivery Mode to reflect the actual interaction:
 - e.g. Telephone, Video Call, etc.

4.13 Using Your webPAS Action List

The webPAS Action List is an essential tool for managing and allocating New and Follow Up outpatient appointments. It displays referrals awaiting scheduling and allows users to filter and sort based on specific criteria, offering a comprehensive view of referral status and urgency.

Key Information Displayed

- Priority: Triage category 1, 2 or 3
- List Days: Number of days the referral has been waiting for a New appointment
- Preferred Date: Date care required
- Visit Type: New or Follow Up
- Appointment Status: Waiting first appt, requested follow up appt, Suspended
- Presenting complaint
- Referral Received Date

Referral Status Definitions

- Waiting: No appointments booked
- Active: Appointment/s booked
- Closed: Patient discharged and referral closed

Referral Allocation Process

- New Referrals: Automatically appear on the specialty Action List once triaged in eReferral. Allocation is typically based on a first-on, first-off approach, considering triage priority and referral preferred date.
- Follow Up Referrals: Scheduled according to the preferred date indicated in the appointment request.

 **Note:** Appointment requests should not be added to Waiting referrals unless the patient is attending multiple clinics within the same specialty (e.g., Rehab Medicine – Consultant Clinic and Falls Clinic).

Daily Review & Follow-Up

Action Lists should be reviewed daily to:

- Update new referrals
- Schedule outstanding New and Follow Up appointments

Patients may still be marked as 'Awaiting First Attended Appointment' even if their referral is Active—this can occur due to:

- Cancellation
- Did Not Attend (DNA)
- Appointment marked as Cancelled/Other (C/O)

These patients must be followed up and rebooked promptly within the appropriate timeframe.

You can identify such cases using the 'New Referral, no Future Booking or Appt Request' tab on the [Outpatient Data Cleansing tool](#)

5. Appointment Notifications

Clerks are responsible for managing various communication tools related to outpatient appointments. It is essential to maintain professionalism and consistency in all communications to ensure efficient and respectful patient engagement.

5.1 Appointment Letters

- All appointments made must have the appointment letter printed immediately by the clerk and given or posted to the patient.
- Australia Post currently take over 7days within the metro area. Any appointments booked within 10 working days from the appointment date must be notified to patients via a telephone call and SMS notification as appointment may not be received in time.

5.2 Telephone Calls

- Patients must be phoned to advise of appointments booked within 10 working days of the appointment date
- Documentation is to be recorded in the webPAS appointment comments field to confirm patient has been contacted.

5.3 Automated SMS Reminders (SMSOR)

The SMSOR system automatically generates and sends SMS reminders to patients to help reduce missed appointments and improve attendance.

Key Points:

- SMS reminders are sent to patients 10 days and 3 days prior to their scheduled appointment.
- Clerical staff are responsible for:
 - Confirming the patient's mobile number at each visit.
 - Verifying with the patient that the SMS reminder was received.



Note: The appointment service delivery mode in webPAS must be accurate each time an appointment is created or modified. This field determines the type of SMS message sent to the patient. Incorrect settings may result in the wrong information being communicated.

5.4 Manual SMS Reminders (Esendex)

Esendex is a manual SMS messaging system used by clerical staff to send customised appointment reminders and notifications to patients. Unlike the automated SMSOR system, Esendex allows for tailored communication in specific scenarios.

Usage Guidelines:

- Messages are sent manually by clerks using pre-approved templates.
- Message wording must be approved by the Outpatient Coordinator to ensure clarity and professionalism.

Common Use Cases:

- Sending appointment details when the patient cannot be reached by phone.
- Urgent Orthopaedic appointments following an ED visit, where time does not permit sending an appointment letter (e.g., appointments with less than 10 days' notice).
- Reminders for patients with a history of Did Not Attend (DNA), sent the day before the appointment.
- Last-minute clinic cancellations, where bulk notifications are required.
- Conversion of appointments from Client Present to Telephone.
- Requesting patients to contact the clinic for waitlist audits or other queries.

 **Note:** All manual SMS communications must comply with the EMHS Clinical Instant Messaging Policy

5.5 Manage My Care App (MMC)

Patients can be advised of and referred to the Manage My Care app, which is available for download on mobile phones and other smart devices. This app provides patients with real-time access to their outpatient referral and appointment information.

Key Features:

- Appointments are displayed in real time, and any changes made in webPAS are instantly reflected in the portal.
- The app helps patients stay informed and engaged in their care by providing up-to-date appointment details.

Important Notes:

- Chart Only appointments, or appointments that have been converted from Chart Only to New or Follow Up, do not appear in the Manage My Care app.
- If a future Chart Only appointment needs to be converted to a New or Follow Up appointment, the original Chart Only appointment must be cancelled, and a new appointment type must be rebooked. Only then will it appear in the app.

5.6 Resources

- [Manage My Care Website](#)
- [Manage My Care FAQ's](#)

6. Reschedule Appointment

Requests to reschedule a patient's appointment may originate from:

- The patient
- The clinician (e.g., due to unavailability)
- The health service (e.g., lockdowns or service disruptions)

Priority Patients

When rescheduling:

- Category 1 New patients
- High Risk (HR) or Time Critical (TC) Follow Up patients

It is essential that these patients are reappointed within their original category or follow-up timeframes.

 Clinician notification is required for any rescheduling of Category 1, HR, or TC patients that will result in appointments being booked outside of recommended timeframes. Use the Rescheduling Cancelling Patient Requests sheet to document and manage these requests in consultation with clinicians. Refer to Appendix 2 for guidance.

Category 2 and 3 Patients

These patients may be rescheduled to the next available appointment, provided they remain within boundary.

If the patient is marked as High Risk or Time Critical, clinician approval must be obtained before making any changes.

Policy Compliance

In accordance with the WA Health Specialist Outpatient Services Access Policy:

- A patient may be removed from the appointment waiting list and have their referral closed if:
 - They reschedule more than twice, or
 - They are unavailable for treatment for 90 days or more, unless prior notice and valid reason is provided.

Hospitals must exercise discretion on a case-by-case basis to avoid disadvantaging patients due to genuine hardship, misunderstanding, or unavoidable circumstances.

Any discharge due to these reasons must be clearly documented in webPAS and the patient's health record. The referral must be discharged, and discharge letters sent to the GP and Referrer.

7. Cancelling Appointment

When to Cancel an Appointment

Appointments should only be cancelled if:

- Treatment is complete
- No further bookings are required

In these cases:

- The patient should be discharged
- The referral must be closed
- The discharge reason must be documented in webPAS

Future Bookings Required but Not Yet Allocated

If future appointments are needed but cannot be scheduled at the time of cancellation:

- Use the Cancel / Pend Reschedule function to place an appointment request on the Action List
- This ensures the referral remains visible until another appointment is allocated
- Include the cancellation date and reason in the appointment request

When No Further Booking Is Required

If the patient is declining treatment or advises they are being treated elsewhere:

- Category 1, 2, High Risk, or Time Critical patients: Escalate to the clinician for advice before proceeding. Refer to Appendix 2.

Patients not in the above categories:

- If being treated privately, escalation is not required; referral can be closed
- If being treated at another public hospital, check webPAS to confirm appointments are booked as advised

Documentation Requirements

- Record cancellation details in the appointment comments field
- Close the referral with the appropriate reason and add a comment
- Issue a discharge letter to the patient's GP and Referrer (if different), advising of the referral closure.

 **Note:** When cancelling appointments in webPAS please record the most accurate description in the webPAS cancellation reason drop down box.

8. Clinic Setup

To ensure clinics run smoothly and efficiently, clerical staff must complete the following tasks in preparation for each clinic day.

8.1 Workday prior to clinic:

- Print clinic lists and one set of patient labels for the following day(s):
 - One list for the clinic clerk
 - One list for each clinician
 - Prepare clinician packs with the clinic list, outcome slips, and patient labels
- Check interpreter requirements and ensure booking is confirmed
- Verify Digital Medical Record (DMR) status in webPAS for each patient, create DMR if required
- Identify any patient alerts and print for clinician
- Ensure all equipment, supplies, and health record forms required for the clinic are available and ready

8.2 Day of Clinic:

- Provide each clinician with their clinic list, outcome slips, and patient labels
- Review upcoming clinics for the next 7 days to ensure all appointment slots are utilised
- Review waiting referrals and the Action List to:
 - Book future appointments in line with triage priority and preferred dates
 - Ensure appointments are scheduled within the booking horizon to allow adequate patient notification
- Complete any additional duties required to support the efficient operation of the clinic

9. Confirming Patient Demographics

Upon presentation to the clinic, all patients must be interviewed to confirm and update their demographic details. This ensures accurate records and appropriate communication.

Steps to Follow:

- Screen for wellness using the Respiratory Infections are Contagious poster at the reception desk, offer patients masks if required to minimise risk to others.
- Confirm and update all Patient Master Index (PMI) fields in webPAS, with the following exceptions:
 - Gender and Pronouns, Ethnicity/Ancestry, and Language fields do not need to be reconfirmed if previously updated.
- For new patients, confirm if care is related to:

- Workers Compensation (WC)
- Motor Vehicle Injury Trust (MVIT)
- Department of Veterans' Affairs (DVA)
- Overseas Visitor (OV)
- Other Specialties (OS)

Ensure the claim type is updated on both the referral and the current appointment.

Address Changes:

- If the patient's residential or postal address has changed:
 - Print new patient labels
 - Provide updated labels to the clinician
 - Update all relevant paperwork for the visit (e.g., outcome slips)

 **Note:** Appointment letters for paediatric patients are sent to Next of Kin address - **confirm this information is current at each visit.** [PMI Standards](#)

10. Arriving Patients

Checking in a patient in webPAS records the exact time the patient presents to the clinic and marks their arrival.

Key Requirements:

- Confirmation of Patient Master Index (PMI) details is mandatory at the time of check-in.
- Once a patient is marked as arrived in webPAS, this status flows through to:
 - Digital Medical Record (DMR)
 - Integrated Clinical Management (ICM)

This allows clinicians to see in real time when patients have arrived in the clinic, supporting efficient patient flow.

11. Claim Type

Claim Types reflect the funding source for a patient's visit and are essential for ensuring accurate billing and appropriate allocation of healthcare costs. All referrals and appointments must have a correctly assigned Claim Type to generate the correct funding and raise invoices for non-Medicare or compensable patients.

When to Confirm or Update Claim Type:

- During referral entry
- At appointment booking
- At check-in, via the Check-In Details screen in webPAS

Claim Type Guidelines

- If the patient has previously attended ED or been admitted for the same injury:
 - Review the previous visit to identify the correct Claim Type
 - Use the spyglass icon next to the claim number to pre-fill all relevant details

Specific Scenarios:

- Workers Compensation / Motor Vehicle Accident (MVIT):

- Claim Type must reflect the compensable nature of the visit
- Clerks must capture all event and insurance details
- Use the spyglass to copy details from ED or ward if referred from there
- Overseas Visitors (OV):
 - Patients without a Medicare number should be recorded as Overseas Visitors
 - If the patient qualifies under another category (e.g., Overseas Student, Reciprocal Health Care Agreement), update accordingly
 - Liaise with the AKG Finance Team to provide current fees and charges
- Corrective Services Patients:
 - Claim Type: Public
 - Funding Source: Corrective Services
 - Remove Medicare details from the patient's demographics
- Reciprocal Health Care Agreement (RHCA):
 - Revenue Department will identify RHCA under the patient's PMI
 - Claim Type: Public
 - Funding Source: Reciprocal Agreement
- Medicare Holders:
 - After confirming the patient does not fall into any other category
 - Claim Type: Public

Resources for Further Guidance:

[webPAS Claim Types Supplementary Information Pack](#)

- [Financial Election Tip Sheet with Flowchart](#)
- [OV/OS Flowchart.](#)



Note: Funding will not be received for appointments that:

- Have a claim type as unknown or no claim type entered
- remain unprocessed
- processed with an outcome entered as unknown

12. Processing Appointments

Following each appointment, clinicians will provide clerical staff with the following information via the clinic list or outcome slip:

- Appointment outcome (e.g., reappoint, discharge)
- Follow-up timeframe or preferred date
- DNA instructions, if applicable

Clerical Responsibilities:

- Enter the patient's departure time in webPAS
- Select the relevant appointment outcome
- When booking a follow-up appointment or adding an appointment request:
 - Transcribe any clinician comments
 - Include any approved communication codes to assist with clinic preparation and reminders

These comments and codes will be visible on future clinic lists.



Note: Patients must be booked into a consultant clinic after attending two consecutive Registrar (Reg) clinic appointments. This ensures appropriate clinical oversight in line with [Outpatient Services Guidelines](#).

Approved Communication Codes:

Code	Meaning
XR	X-ray
US	Ultrasound
CH	Chaperone
PW	Pathology
TC	Time Critical
EQ	Equipment (e.g., Moon boot)
HR	High Risk

Timeliness of Processing:

- Appointments must be processed in webPAS immediately following the appointment or by the end of the clinic session
- Clerks are responsible for ensuring all outcomes are obtained from clinicians before the clinic ends
- If this is not possible, outcomes must be entered within 48 hours of the appointment

 **Note:** All appointments must be entered and processed by the 4th of the following month to ensure inclusion in patient activity reporting

13. Outcoming Patient Events

Only processed appointments with valid outcomes are counted and funded. Completing the appointment outcome in webPAS triggers the service event to flow through to the patient billing systems, ensuring accurate revenue capture—particularly for overseas and compensable patients.

13.1 Outcome Options

Chart Only	✗ Do not use. Ensure actual outcomes are entered that reflect the patient journey
Discharge from Clinic	✓ Treatment complete; patient discharged to GP. Also used for DNA discharges per policy. Referral must be closed (except for multidisciplinary services like DIAB and CR).
Pt Did Not Attend (DNA)	✓ Patient did not arrive. May require the referral to be closed after clinical review. Book next appt or discharge before moving onto next patient so pt is not lost to follow up.

Pt Left Not Seen	☒ Patient arrived but then elected not to wait or patient advised clinician unavailable. Enter reason in comments e.g. clinician unavailable due to emergency.
Pt Late not seen	☒ Do not use. Patient should be seen unless clinic is over – then DNA is appropriate.
Reappoint	☒ Patient requires follow-up. Book appointment or add request if beyond 90-day horizon.
Refer IP Waitlist	☒ The patient is being waitlisted for surgery. Depending on follow up process for your clinic, referral may be closed at this point and reopened when post op booking is required.
Outcome Unknown	☒ Do not use. Follow up with clinician to obtain correct outcome.
Pt Deceased in Op Clinic	☒ Referral to be closed.
Refer other specialty	☒ Do not use. Select Discharge from Clinic, close referral, and note "Referred to [Specialty]" in comments.
Refer other hospital	☒ Do not use. Select Discharge from Clinic, close referral, and note "Referred to [Hospital]" in comments.
Refer to ED	☒ Do not use. Reappoint or discharge as appropriate.
Refer back to GP	☒ Do not use. Select Discharge from Clinic, close referral, and note "Referred back to GP" in comments.
Return to Ward	☒ Do not use. Reappoint or discharge as appropriate.
Admit to Ward	☒ Referral may be closed and reopened post-discharge depending on clinic process.

13.2 SOS Discharges:

Patients have 3-6 months ability to call and request SOS follow up appointment for issues related to original issue treated only. If unsure whether referral should be reopened check with the clinician/nurse first.

14. Did Not Attend (DNA) Process

A Did Not Attend (DNA) occurs when a patient fails to attend their scheduled appointment without prior notice or attempt to cancel/reschedule before the appointment time.

14.1 Clinician Action:

- Clinicians should use the allocated appointment time to attempt contact with the patient.
- If contact is successful and the consultation meets the criteria for a service event, the appointment may be updated to a telephone consult.

14.2 High-Risk Patients:

For Category 1 New, High Risk (HR), or Time Critical (TC) patients:

- Clerical staff must liaise with the clinician to ensure telephone contact is attempted to mitigate clinical risk.
- If a telephone consult does not occur, any subsequent bookings must be made within the original category or follow-up timeframes.

14.3 Discharge Criteria (DoH Specialist Outpatient Services Access Policy):

Patients may be discharged from the clinic if:

- They fail to attend their initial (New) outpatient appointment, and the clinician deems the referral non-urgent
- They have two consecutive DNAs for follow-up appointments under the same referral
- They have two consecutive reschedule requests or are unavailable for more than 90 days, unless prior notice and valid reason is provided

Clinicians may request reappointment on a case-by-case basis to avoid disadvantaging patients due to genuine hardship, misunderstanding, or unavoidable circumstances. This is by exception only.

14.4 Clerical Responsibilities:

- Book a follow-up appointment or add an appointment request as directed by the clinician
- Referrals must not be left open without action
- If the patient contacts the clinic after being discharged, the referral can be reopened

15. Retrospective Data Entry Clinics

Retrospective data entry is approved for specific clinics where appointments are fluid, urgent, or where clerical support is unavailable. These clinics have designated timeframes for submitting data sheets to ensure outpatient attendances are accurately captured and reported.

Clinic	Data Sheet Deadlines	Clinical Contact Position
Maternity Ward Postnatal and Telephone (not AAU)	Daily	Maternity Manager
VMS	Daily	Assistant Midwifery Manager
Lactation	Daily	ANC Coordinator
Newborn Hearing	Daily	ANC Coordinator
Social Work	Weekly	SW
ANC Psychiatric Liaison Nurse	Monthly	PLN
Dietetics HEN	Weekly	Dietetics Coordinator
Rehab Home Enteral Nutrition (HEN)	Monthly	Therapy Assistants
Paeds Ward Telephone calls/ c/o	Daily	Clerk

Process Overview:

- Data Entry Sheets must be completed for each clinic listed above. Refer to Appendix 3.
- Sheets are to be submitted to the relevant Clinic Coordinator or Contact Person, who will:
 - Monitor completion for accuracy
 - Cross-check against staff rosters ensuring all sheets are accounted for
- The Clinic Contact will forward completed sheets to the clerical team for data entry as per the above schedule.
- End-of-month data sheets must be submitted by the 1st of the following month to allow data entry to be completed before the 4th.

 **Note:** All appointments must be entered and processed by the 4th of the following month to ensure inclusion in patient activity reporting.

15.1 HEN Retrospective Data Entry Clinics

The HEN Clinic operates under a nationally approved exception to standard outpatient data entry rules. It is the only clinic where a Client Present appointment is added once per month for each HEN patient, even if no face-to-face or telephone consultation has occurred.

This process is approved Australia-wide to ensure appropriate funding is received for script preparation and ordering of HEN supplies completed by Dietitians

Key Points:

- This exception applies only to the HEN Clinic.
- The appointment is added monthly in webPAS using the Client Present visit type.
- No direct patient contact is required for the appointment to be valid.

Resources for Further Guidance:

[AHS Recording Instructions for Home Enteral Nutrition Self Care](#)

16. End of Clinic Session

To ensure accurate documentation, timely processing, and smooth clinic operations, clerical staff must complete the following tasks at the end of each clinic session.

16.1 Appointment Outcomes

- Follow up with clinicians to obtain any outstanding appointment outcomes before they leave the clinic.
- Ensure DNA outcomes are reviewed, outcome slips are completed, and appointments are processed accordingly.
- Review the [Rescheduling Cancelling Patient Requests sheet](#) with clinicians and ensure all items are actioned.

16.2 Medical Dictations

- Clinicians complete dictations using the National Transcription Service (NTS) via mobile device or consult room landline.
- Medical Typists transcribe dictations using InQuery and manage distribution of letters to addressees.
- Finalised letters are automatically uploaded to:
 - DMR
 - iCM
 - My Health Record
- For transcription queries or access requests, contact the Transcription Coordinator AKG.Transcriptions@health.wa.gov.au

16.3 Pre prepping loose document for scanning into patient medical record

16.3.1 Clerical staff must ensure:

- All documents have patient labels or clearly handwritten UMRN on the top right corner.
- Paperwork is repaired using invisible tape (no folds, rips, or tears).

- Multiple pages for the same patient are bulldog clipped together.
- Documents are placed in yellow medical record wallets and place in designated DMR collection tray.

16.4 Returning Health Records (if applicable)

16.4.1 Clerical to:

- Scan health records and place them in terminal digit order in the trolley.
- Return to the Medical Records Department.
- If clinicians still require 1–2 records, contact the After-Hours Supervisor on extension 2073 to arrange collection and return once the clinician is finished.

16.5 Handover

- Provide a thorough handover via email to colleagues, outlining:
 - Any outstanding tasks
 - Any urgent follow-ups required the next day
- This is essential if you will not be in clinic the following day.

17. Clinic Duties

17.1 Telephones

All telephone interactions must align with the EMHS Phone Etiquette Guidelines. Staff are expected to manage calls professionally and courteously, ensuring patient confidentiality and efficient communication.

**Government of Western Australia
East Metropolitan Health Service**

EMHS Etiquette

- Kindness**
- Respect**
- Collaboration**
- Integrity**
- Accountability**
- Excellence**

Answer telephone call in 3 rings

- answer the call with a smile in your voice
greet the caller with your name and department
- ask the caller's permission before placing their call on hold transferring;
seek the caller's permission to place their call on speaker phone
- provide the receiver of the transferred call, details of the caller (their name) and why they are calling
- avoid being interrupted while speaking with the caller
- once the initial query is resolved, ask the caller if there is anything else you can help them with
- thank the person for calling

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17.2 Emails

Email communication must be handled with diligence and professionalism:

- **Mailbox Management:** Both personal and generic mailboxes must be maintained consistently.
- **Patient Communication:** Emails to patients must be sent from generic mailboxes only. Personal accounts must not be used.
- **Information Restrictions:** No clinical patient information is to be sent outside the health system via email. Only generic procedural information (e.g., surgical handouts, brochures, appointment letters) may be shared externally.
- **Email Practices:**
 - Address responses appropriately and to the correct audience.
 - Prioritise emails as needed.
 - Flag items for follow-up and unflag once complete.
 - Move completed items to the 'Completed' folder in generic inboxes.
 - Do not delete emails from generic inboxes to ensure auditability and accessibility for other staff.

17.3 Mail

Mail must be collected from designated clinic drop-off locations throughout the day and placed in the appropriate clinician trays/pigeonholes for collection during clinic hours.

17.4 Stock Control

Outpatient Clerks are responsible for maintaining adequate supplies of stationery and medical record forms, including:

- MR5 Outpatient Notes (utilised during DMR downtime only)
- Radiology Request Forms
- Script Pads
- Medical Certificates
- Consent Forms
- Any other clinic-specific forms

17.4.1 Ordering Procedures:

- Medical record forms: Ordered via PMG fortnightly (1st and 15th of each month) or retrieved from Stores.
- Stationery and paper: Ordered directly through COS.
- Envelopes: Collected from Stores.
- Printer cartridges/toners: Email order requirements (including toner type and cost centre) to After Hours Clerical Supervisor at: AHS_AfterHoursClerical@health.wa.gov.au

17.5 Outpatient Data Cleansing Tool

The [OP reform data cleansing tool - Power BI](#) is designed to identify:

- Over-boundary referrals
- Data entry errors
- Patients no longer visible on action lists

The cleansing tool allows users to drill down to patient level data, export and filter as required.

17.5.1 Usage Guidelines:

- Review and action discrepancies daily.
- The dashboard updates at midnight and supports accurate data capture for patient care and funding.
- Weekly review and correction of data is mandatory.
- Top 3 audit items must be actioned by the 3rd of each month for reporting inclusion.

17.5.2 Audit Reports Include:

- Unprocessed appointments (attended with no outcome)
- Outcome marked as 'unknown' (must be updated)
- Over-boundary referrals not booked (must be booked ASAP)
- Outcome reappoint but no appointment booked or appt request added to referral
- Incorrect chart-only processing
- Duplicate referrals in webPAS
- Unknown claim types in appointments or referrals (must be corrected)

 **Note:** Data is to be reviewed and corrected Daily. Top 3 dot points must be actioned by 3rd of each month for reporting inclusion.

17.6 Clinicians Leave

When informed by clinician that they are taking leave:

- Reserve slots in webPAS noting "clinician unavailable"
- Notify AHS.outpatientclinicmanagement@health.wa.gov.au and AKG.webpassupport@health.wa.gov.au if beyond horizon.

18. Roles and responsibilities

Role	Responsibility
Outpatient Clinic Clerk	All duties outlined in this document to ensure efficient, safe provision of outpatient services.
Outpatient Coordinator	Daily liaison with outpatient clinic staff, weekly review of outpatient data to ensure efficient, safe provision of outpatient services.
Manager, HMS	Regular liaison with Outpatient Clerical Coordinator to ensure efficient, safe provision of outpatient services, resolution of issues and monitoring of outpatient data.

19. Legislative context

- HDWA
 - [Non-Admitted Patient Activity Data Business Rules](#)
 - [Specialist-Outpatient-Services-Access-Policy](#)

20. Supporting information

The following documents support the implementation of this policy:

- [Interpreter and Translator Guidelines](#)
- [webPAS User Guides](#)

- [AHS Recording Instructions for Home Enteral Nutrition Self Care](#)
- [OP reform data cleansing tool - Power BI](#)
- [Outpatient Services Guidelines](#)
- [PMI Standards](#)

21. Compliance, monitoring and evaluation

Compliance against this guideline will be undertaken by Outpatient Clerical Coordinator, webPAS System Administrator, Manager HIMS and Manager Outpatients through regular liaison with outpatient clerical staff and review/auditing of outpatient data.

Outpatient data cleansing tool and activity reporting so be reviewed monthly as a minimum and issues identified followed up with relevant clerical and clinical staff.

22. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

Standard 1 – Action 1.1 6 Support the workforce to maintain accurate and complete healthcare records.

23. Document owner

Director Operations and Corporate Services.

Enquiries relating to this guideline may be directed to:

Title: Manager, Health Information Management Service

Department: Health Information Management Service

Email: AHS.HIMSManager@health.wa.gov.au

24. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V2	01 12 2025	01 12 2028	Full Review; updated process for DNA and links and addition of SOS Discharges.

25. Authorisation

This guideline has been authorised and issued by:

Authorised by	Michael Morton - Director Operations and Corporate Services.
Authorisation date	01 12 2025
Published date	01 12 2025

26.2 Appendix 2: Retrospective Data Entry Sheet

Discipline _____ Name of clinician _____

SERVICE EVENT RECORDING SHEET											
Date & Times	UMRN	Patient Name	Service Type NEW, FOL, HNV, GRP C/O	Clinic Name & Code	Delivery Mode C=Client Present T=Telephone V=Video Call D= DNA C/O = Chart review	Outcome 1) Reappoint • F2F • Tel • Vid • C/O 2) Discharge	Documented in Patient Record Yes/Tick	Date of next Appt & Clinic code	Financial Election Public. Workers Comp Motor Vehicle DVA Overseas Other	Comments HR: High Risk TC: Time Critical CH: Chaperone	WebPAS Data Entry Done Date and initial

"Service Event" definition:

A non-admitted patient service event is defined as an interaction between one or more healthcare providers with one non admitted patient, which must contain therapeutic/clinical content and result in a dated entry in the patient's medical record.